QUILLMD

# Dr. Smoke Tester MD · Primary Care

| PATIENT | VISIT DATE | CHIEF COMPLAINT |
|---|---|---|
| Smoke TestPatient<br>OneCardiology<br>1777863896718 | 2026-05-04 | Chest pain x 2 hours, radiation to left arm |

## CLINICAL NOTE

### Subjective

**Chief Complaint:** Chest pain that started two hours ago.

**HPI:** The patient is a 58-year-old male presenting with substernal chest pain of two hours' duration. He describes the quality as crushing, with radiation to the left arm. Associated symptoms include shortness of breath and nausea. Pain severity (0–10 scale), onset context (rest vs. exertion), and modifying factors (aggravating/relieving factors) were not documented at time of encounter; given the acuity of presentation and expedited transfer, these elements were not obtainable — noted explicitly for ED team follow-up. Presence or absence of diaphoresis was not assessed during this encounter; ED team should evaluate upon arrival.

**PMH:**

- Hypertension
- High cholesterol

**Medications:**

- Lisinopril 20 mg oral daily
- Atorvastatin 40 mg oral daily

**Allergies:** Allergy status unknown at time of encounter. Aspirin and nitroglycerin were administered; flag for ED handoff and clarification.

**Family History:** [not documented]

**Social History:** [not documented]

**ROS:**

• *Cardiovascular:* Chest pain present (see HPI for full characterization); no syncope or palpitations documented.
• *Respiratory:* Shortness of breath present; no pleuritic component documented.
• *Gastrointestinal:* Nausea present; no vomiting documented.
• *Constitutional:* Diaphoresis not assessed at time of encounter — to be evaluated by ED team; no fever reported.
• *Neurological:* No focal neurological symptoms documented.

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### Objective

**Vital Signs:**

- Blood pressure: 158/92 mmHg
- Heart rate: 88 bpm
- Oxygen saturation: 96% on room air

- Respiratory rate: not measured at time of encounter
- Temperature and weight: [not documented]

**Cardiovascular:** Heart sounds regular; no murmur appreciated.

**Respiratory:** Lungs clear to auscultation.

All other systems: [not documented in this encounter]

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### Assessment

**Primary Clinical Consideration:** Acute coronary syndrome (ACS) — rule out.

**Supporting Reasoning:** 58-year-old male with known cardiovascular risk factors (hypertension, hyperlipidemia) presenting with acute-onset crushing substernal chest pain radiating to the left arm, accompanied by shortness of breath and nausea. Vital signs are notable for elevated blood pressure at 158/92 mmHg. Cardiac auscultation is unremarkable; lungs are clear. The clinical presentation is highly concerning for an acute ischemic event pending confirmatory workup.

**Differential Diagnoses:**

- **Unstable angina** — high on differential given risk factor burden and symptom character; distinction from NSTEMI requires troponin and EKG correlation.
- **Aortic dissection** — must be considered given hypertension and severity of presentation; notably, aspirin and nitroglycerin have already been administered, which carries risk if dissection is confirmed — ED team should evaluate urgently with appropriate imaging (e.g., CT angiography) as clinically indicated.
- **Pulmonary embolism** — pleuritic chest pain and hemoptysis are absent per available history, making this less likely, but cannot be excluded without further workup.
- **Esophageal spasm** — can mimic ACS with substernal crushing quality; lower priority given risk factor profile but remains on differential pending workup.
- **GERD/esophageal etiology** — less likely given acuity and associated symptoms but noted for completeness.

ACS (including unstable angina and NSTEMI) remains the leading concern given the constellation of symptoms, risk factors, and hemodynamic findings.

**Severity/Staging:** [not documented]

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### Plan

1. **Medications — Initiated:** Aspirin 325 mg chewed (administered in office). *Note: Allergy status unknown at time of administration — flag for ED team.*
2. **Medications — Initiated:** Sublingual nitroglycerin (administered in office). *Note: Use of PDE-5 inhibitors (e.g., sildenafil, tadalafil, vardenafil) — an absolute contraindication to nitroglycerin — was not formally assessed or documented prior to administration; ED team should clarify PDE-5 inhibitor use as part of medication reconciliation.*
3. **Diagnostics — Ordered:** 12-lead EKG.
4. **Diagnostics — Ordered:** Troponin level.
5. **Disposition:** Transfer to emergency department for further workup and management of suspected acute coronary syndrome. ED handoff should include: clarification of allergy status, PDE-5 inhibitor use, pain severity/modifying factors, and assessment of diaphoresis.

## DIAGNOSES

**R07.9** Chest pain, unspecified
**I10** Essential (primary) hypertension
**E78.5** Hyperlipidemia, unspecified
**R06.00** Dyspnea, unspecified
**R11.0** Nausea

## PROCEDURES / E&M

**99215** Established patient, high complexity medical decision making
**93000** Electrocardiogram, routine ECG with at least 12 leads; with interpretation and report

*Modifier -25: A separately identifiable, high-complexity E&M service (99215) was performed on the same date as the 12-lead EKG (93000). The E&M is driven by the acute ACS evaluation, differential diagnosis formulation, medication administration, and ED disposition — all distinct from the technical performance and interpretation of the EKG itself. Evidence: 'EKG, and troponin... Transfer to emergency department for further workup' alongside the full history, exam, and MDM documented in the SOAP note.*

## MEDICATIONS

Aspirin 325 mg tablet – Chew 1 tablet (325 mg) once — administered in office (#1, Refills 0)
Nitroglycerin sublingual tablet – 1 tablet sublingually as needed for chest pain — administered in office
Lisinopril 20 mg tablet – Take 1 tablet by mouth once daily [CONTINUE]
Atorvastatin 40 mg tablet – Take 1 tablet by mouth once daily [CONTINUE]

**Rx safety warnings — review before sending.**

**[CRITICAL]** Aspirin — Allergy status is unknown at time of administration. Aspirin was given before allergies could be confirmed. ED team must clarify allergy history immediately upon patient arrival — including aspirin/NSAID hypersensitivity and aspirin-exacerbated respiratory disease (AERD). If a relevant allergy is identified, adverse reaction management may be required.
**[CRITICAL]** Nitroglycerin — PDE-5 inhibitor use (sildenafil, tadalafil, vardenafil) was NOT assessed prior to nitroglycerin administration. Concurrent use of nitroglycerin with PDE-5 inhibitors is an absolute contraindication due to risk of severe, potentially fatal hypotension. ED team must urgently clarify PDE-5 inhibitor use as part of medication reconciliation.
**[CRITICAL]** Nitroglycerin — Aortic dissection is listed as an active differential diagnosis. Nitroglycerin has already been administered. If aortic dissection is confirmed on ED workup, nitroglycerin use carries significant risk (reflex tachycardia, hemodynamic instability). ED team should urgently evaluate with appropriate imaging (e.g., CT angiography) and reassess nitroglycerin safety.
**[CRITICAL]** Aspirin — Aortic dissection is listed as an active differential diagnosis. Aspirin has already been administered. If aortic dissection is confirmed, antiplatelet therapy may worsen hemorrhagic complications. ED team should urgently evaluate with appropriate imaging and reassess.
**[MINOR]** Nitroglycerin — Sublingual nitroglycerin strength (dose in mg) and number of doses administered in office were not documented. Standard dosing is 0.3–0.4 mg SL; please confirm dose and number of administrations for accurate ED handoff and medication reconciliation.
**[MINOR]** Aspirin — Allergy status unknown at time of administration — flag for ED handoff and clarification.

## REFERRALS

**→ Emergency Department** (stat)
Transfer for urgent evaluation and management of suspected acute coronary syndrome. Further workup including serial troponins, 12-lead EKG interpretation, and advanced imaging (e.g., CT angiography if aortic dissection remains on differential) is required beyond the scope of the current setting.

*58-year-old male with hypertension and hyperlipidemia on lisinopril 20 mg daily and atorvastatin 40 mg daily, presenting with two hours of crushing substernal chest pain radiating to the left arm with associated shortness of breath and nausea. BP 158/92 mmHg, HR 88, SpO2 96% on room air; cardiac exam unremarkable. Aspirin 325 mg chewed and sublingual nitroglycerin administered in office; allergy status unknown and PDE-5 inhibitor use not formally assessed —*

*both require clarification on arrival.*

## PATIENT SUMMARY

What we talked about today
You came in today because you have been having chest pain for the past two hours. You told us the pain feels crushing and is in the center of your chest. It also spreads to your left arm. You have also been feeling short of breath and sick to your stomach. We took this very seriously and acted quickly.

What we found
We checked your vital signs. Your blood pressure was higher than normal at 158/92. Your heart rate and oxygen level were in an acceptable range. We listened to your heart and lungs. Your heart sounded regular with no unusual noises. Your lungs sounded clear. Your symptoms and history are very concerning for a possible heart problem. You also have a history of high blood pressure and high cholesterol, which raises your risk.

What we're doing about it
Because your symptoms could mean your heart is not getting enough blood, we acted right away. We gave you aspirin to chew and a small tablet under your tongue to help open up your blood vessels. We ordered a heart-tracing test and a blood test to check for signs of heart damage. We are sending you to the emergency department so you can get the full care and testing you need as quickly as possible. The emergency team will follow up on everything and keep working to find out exactly what is going on.

Your medications
You currently take lisinopril (a blood pressure medicine) 20 mg by mouth once a day and atorvastatin 40 mg by mouth once a day for high cholesterol. We gave you aspirin and a nitroglycerin tablet today as part of your urgent care. The emergency team will review all of your medicines and make any needed changes.

Your next steps
You are being transferred to the emergency department right now. Please go directly and do not delay. The emergency team has been told about your visit and what we found. They will continue your care, run more tests, and decide on the best treatment for you.

When to call us
Because you are going to the emergency department right now, that team is your first point of contact. However, if for any reason you are not able to get there or your symptoms get worse on the way, call 911 immediately. Watch for any of these warning signs:
- Chest pain that gets worse or spreads
- Severe headache or sudden vision changes
- Feeling faint or passing out
- Trouble breathing that gets worse
- Pain spreading to your jaw, neck, or back
- Feeling very sweaty or clammy

Do not wait. Call 911 right away if any of these happen.

Take care,

Not yet signed.